TE-03 Drowning / Submersion Injury

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026

Rescue safety and ventilation come first. Drowning is a hypoxic emergency. Do not create an additional victim or delay effective breaths and CPR for routine interventions.

Recognition and immediate field flow

Step	Approved action
1. Safe rescue	Only trained and equipped rescuers enter hazardous water. Request water-rescue resources when needed. Trained responders may provide in-water breathing support only when safe and without delaying removal.
2. Initial breaths	For apnea or unresponsiveness, provide two effective initial breaths/ventilations by the first effective means available. Do not delay for routine suctioning.
3. Pulse?	No pulse: begin CPR with breaths and compressions, then follow the age-appropriate cardiac-arrest pathway. Pulse present: continue oxygenation and airway support.
4. Trauma?	Use spinal-motion restriction only when a traumatic mechanism or findings indicate it. Never delay oxygenation, ventilation, or CPR.
5. Dry and monitor	Remove wet clothing, dry and warm the patient, assess glucose, ECG, temperature, lung findings, oxygenation, work of breathing, and mental status.
6. Transport	Encourage evaluation after any symptomatic event. Transport for cough, foam, dyspnea, abnormal lung sounds, hypoxia, abnormal vital signs, altered mental status, or any clinical concern.

Provider-level actions

Level	Approved actions
EMT	Prioritize rescue safety; provide two initial breaths/ventilations when indicated; ventilate through foam without routine suctioning; begin CPR with breaths and compressions if pulseless; dry and warm the patient.
AEMT	All EMT care plus IV/IO access and authorized treatment of glucose abnormality, shock, or associated hypothermia.
Paramedic	All prior care plus advanced airway/cardiac monitoring and consultation for suspected dive-related barotrauma or decompression illness.

Critical safeguards

- Do not waste time suctioning airway foam. Ventilate through it; suction water or vomit only when needed to clear the airway.

- Do not use a single submersion-time cutoff alone to decide whether to begin or continue resuscitation.

- Use the applicable cardiac-arrest and termination pathway with the complete clinical context and early Medical Control consultation when needed.

Airway and ventilation

Topic	Required action
Ventilation	Give oxygen and effective positive-pressure ventilation when indicated. Verify visible chest rise and reassess respiratory effort, lung sounds, oxygenation, and mental status frequently.
Cardiac arrest	CPR after drowning includes breaths and chest compressions. Apply an AED/defibrillator after CPR with breaths is underway; do not delay ventilation or compressions while setting up the device.
Foam and emesis	Foam is common and is not an indication for prolonged suction attempts. Clear obstruction, vomit, or water when it prevents effective ventilation.
Dive concerns	For a diving mechanism, assess for chest pain, neurologic symptoms, decompression illness, barotrauma, or arterial gas embolism and notify the receiving facility early.

Transport and handoff

- Report water type, estimated submersion circumstances, rescue interval, witnessed status, initial condition, initial breaths/CPR, temperature exposure, trauma mechanism, and changes in respiratory or neurologic status.

- Document oxygenation, ventilation, ECG, glucose, temperature care, airway procedures, response to care, and whether water-rescue or dive resources were involved.

- Use the applicable pediatric or adult airway, cardiac-arrest, hypothermia, trauma, and shock protocols as indicated.

Critical reminder: Effective ventilation is central to drowning resuscitation. Routine spinal restriction, copious suctioning, or a presumed prolonged submersion interval must not displace lifesaving airway, breathing, and circulation care.

Release note

- This direct source PDF replaces the prior imported sheet and matches Claiborne-approved TE-03 app content reconciled October 30, 2026.